Board Memo, Memory Care Investment Request FY27

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To: Board Finance and Strategy Committee. **From:** Strategy and Planning Office. **Date:** 2026-09-24. **Decision requested:** approve Option A planning and capital for the Memory Care service line.

Summary

Demand for dementia diagnosis is outgrowing capacity, and anti-amyloid therapies for early Alzheimer's disease are now part of the competitive landscape. Management recommends Option A: launch an anti-amyloid infusion program at West Campus. Estimated capital and start-up cost is $1.8M.

Situation

- New memory clinic referrals reached 2,430 in FY2026, up 22% from 1,990 in FY2025.
- The median wait from referral to a new memory clinic visit is 94 days, against a 30-day target.
- About 140 patients per year could be eligible for anti-amyloid therapy after screening, amyloid confirmation and MRI safety checks.
- Our 38 infusion chairs ran at 86% utilization in Q3 2026; North Campus cannot absorb new demand without displacing oncology.
- The service area has about 412,000 residents aged 65 and older, projected to grow 18% by 2032.
- Lakeshore Neuro Institute (2024) and Meridian Memory Partners (2025) already offer anti-amyloid infusion; Prairie Valley Health does not.

Options compared

	Option A: West Campus program	Option B: partner with Lakeshore
Capital and start-up	$1.8M	Low
Infusion chairs	3 dedicated at West Campus	None (Lakeshore infuses)
MRI	About 880 added MRIs in year one	Monitoring split across systems
Revenue	Infusion and MRI revenue retained	Infusion revenue lost
Patient impact	Care stays local	Longer drive for many patients

Option C, deferring 12 months, has the lowest cost but is likely to increase referral leakage to competitors. We estimate 180 to 220 of our referred patients a year already go to Lakeshore.

Why Option A over Option B

Option A keeps the biomarker-confirmed, MRI-monitored patient population inside Northlake, which is the same capability trial sponsors screen for. Option B depends on a competitor that already reports a waitlist for evaluation, so it does not solve access for our patients.

Risks and mitigations

- Recruitment: 2 neurologists usually take 9 to 12 months to recruit. Mitigation: advanced practice providers for follow-up visits while recruiting.
- MRI access: without protected slots, safety MRIs could delay infusions. Mitigation: 6 protected 3T slots per week and a 24-hour read target for ARIA-protocol scans.
- Reimbursement: CMS covers these drugs under coverage with evidence development, which requires a qualifying registry. Missing registry data or MRI documentation puts payment at risk. Mitigation: a registry-submission workflow and 1 authorization specialist.
- Caregiver burden: patients rely on care partners for frequent visits. Mitigation: apply to the CMS GUIDE Model in the next cohort; Northlake has not yet joined.

The ask

Approve Option A planning and release the $1.8M in two tranches: $1.1M on approval for West Campus chair preparation, the MRI protocol build and recruitment, and $0.7M after the go/no-go review at the end of Q2 FY2027.

Conditions carried from the strategy memo: fix the diagnostic bottleneck first, confirm MRI read turnaround for safety scans, and apply to the GUIDE Model.